Patient conversation

Patients need support, not the burden of a stress-free pregnancy

SUGGESTED COUNSELING LANGUAGE

"You do not have to achieve a stress-free pregnancy. You deserve support with what you are facing."

The goal is to make life more manageable and treat distress.

Sources R1 R4

Patient conversation

Ask about distress and connect screening to care

01

Ask

How are you coping?

→

02

Screen

Validated anxiety / depression tools

→

03

Assess

Symptoms, function, and safety

→

04

Act

Treatment, support, and follow-up

> Screen early in pregnancy, later in pregnancy, and postpartum. Screening must connect to care.

Sources R5

Patient conversation

Practical support starts with the stressor the patient identifies

Daily burdens

Food • housing • transport
Work • caregiving

Relationships and safety

Trusted support
A private conversation about safety

One practical step

Choose a concrete task
Connect to someone who can help

Sources R4

Patient conversation

Small routines may help coping without becoming another obligation

Choose an option to discuss with your care team.

- A sleep routine
- A quiet breathing break
- A supportive conversation

A small step is enough to start. Nothing is saved or sent.

> These are coping supports. They have not been shown to reverse this study’s placental changes.

Sources R4

Patient conversation

Movement can support wellbeing when medically appropriate

150 min

moderate aerobic activity per week

For uncomplicated pregnancy

Start gradually. Ask your clinician what is appropriate for your pregnancy.

> General ACOG pregnancy guidance; not a treatment tested in this study.

Patient conversation

Therapy has stronger clinical support than promises to block stress transmission

Treatment

Talk therapy can help with anxiety and daily functioning.

Prevention

Cognitive behavioral therapy or interpersonal counseling can help prevent perinatal depression in people at increased risk.

Evidence boundary

Neither proves prevention of this paper’s proposed fetal-brain effects.

Sources R4 R7

Patient conversation

Medication decisions require an individualized risk–benefit discussion

01

Review

Symptoms and previous treatment response

→

02

Discuss

Medication-specific risks and untreated illness

→

03

Decide together

A plan with your treating clinicians

> Do not abruptly stop a prescribed medicine because of this headline.

Sources R4 R8

Patient conversation

Persistent impairment or immediate danger needs more than self-care

Support

Share stressors and arrange follow-up.

Prompt assessment

Symptoms interfere with eating, sleeping, work, or daily life.

Urgent assessment

Self-harm thoughts, psychosis, or inability to stay safe. Seek emergency help for immediate danger.

Sources R5 R8

Patient conversation

A one-minute conversation can replace guilt with a plan

SUGGESTED COUNSELING LANGUAGE

"The brain findings were in mice. The human part looked at placentas, not babies' brains or later learning. This cannot tell us that your stress harmed your baby. Your wellbeing matters. What is hardest right now, and what support would help?"

Agree on one next step and a follow-up plan. Ask the patient what they will take away.

Sources R1 R4 R5

Patient conversation

Ask your MFM specialist for a plan you can use

1. Do my symptoms need treatment?
2. Who can help with practical burdens?
3. What activity is appropriate for me?
4. Should my medicines change?
5. When will we check in again?

General education, not an individual diagnosis or a substitute for care. This presentation collects no personal information.

Source register

References: screening, counseling, and activity

R4 · PATIENT GUIDANCE

American College of Obstetricians and Gynecologists. Anxiety and Pregnancy. Accessed October 23, 2026.

R5 · SCREENING GUIDELINE

American College of Obstetricians and Gynecologists. Screening and Diagnosis of Mental Health Conditions During Pregnancy and Postpartum. Clinical Practice Guideline No. 4. Obstet Gynecol. 2023;141:1232–1261. Official implementation summary linked.

R6 · ACTIVITY GUIDANCE

American College of Obstetricians and Gynecologists. Physical Activity and Exercise During Pregnancy and the Postpartum Period. Committee Opinion No. 804. Obstet Gynecol. 2020;135:e178–e188. See also the current Exercise During Pregnancy patient guidance.

Source register

References: prevention, treatment, and review limits

R7 · PREVENTION RECOMMENDATION

US Preventive Services Task Force. Perinatal Depression: Preventive Interventions. Final recommendation statement. February 12, 2019. Grade B. Update in progress as of review.

R8 · TREATMENT GUIDELINE

American College of Obstetricians and Gynecologists. Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum. Clinical Practice Guideline No. 5. Obstet Gynecol. 2023;141:1262–1288.

Review: October 23, 2026. Full supplied study reviewed; raw data and standalone supplements not audited. Some guideline full text was access-restricted. Clinical recommendations use accessible official guidance. General education; no personal data collected.